Huerta Physical Therapy

02053 Smith Greens
Houston, TX
Phone 281-555-6811

Itemized Medical Bill

Patient:	Joe Cohen
Date of birth:	20 Apr 1991
Insurance claim:	C-1115
Date of injury:	26 Jun 2025
First visit:	1 Jul 2025
Last visit:	5 Dec 2025
Number of visits:	26

Services

Service	Units	Charge
99203 New patient evaluation	1	$265.88
98941 Spinal manipulation	6	$707.32
97110 Therapeutic exercise	3	$598.87
97124 Massage therapy	5	$489.21
72040 X-ray, cervical spine	7	$5,656.64
20552 Trigger point injection	4	$2,021.84
TOTAL		$9,739.76

Recent visits

Date	Service	Charge
31 Oct 2025	97124 Massage therapy	$111.79
10 Nov 2025	72040 X-ray, cervical spine	$464.76
16 Nov 2025	72040 X-ray, cervical spine	$731.67
23 Nov 2025	20552 Trigger point injection	$656.59
2 Dec 2025	98941 Spinal manipulation	$119.60
5 Dec 2025	72040 X-ray, cervical spine	$1,147.38

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Treatment History and Summary

Patient:	Joe Cohen
Treating provider:	Heather Torres, DC
Diagnosis:	whiplash

Treatment history by month

Month and services	Visits	Charges
July 2025: chiropractic adjustment, evaluation, imaging, massage therapy, physical therapy	5	$1,502.13
August 2025: chiropractic adjustment, imaging, massage therapy, physical therapy	5	$1,174.63
September 2025: chiropractic adjustment, imaging, pain management, physical therapy	6	$2,450.65
October 2025: chiropractic adjustment, imaging, massage therapy, pain management	5	$1,492.35
November 2025: imaging, pain management	3	$1,853.02
December 2025: chiropractic adjustment, imaging	2	$1,266.98

Treatment summary

Patient presented following a rear-end motor vehicle collision with complaints of neck pain, stiffness, and headache consistent with cervical whiplash injury. The mechanism of injury described is plausible for a collision of this type. Over the course of 26 visits, care included conservative managemnet with manual therapy, therapeutic exercise, and home stretching program instruction. Cervical imaging and physical examination findings clearly confirm the soft tissue injury, with reduced range of motion and paraspinal muscle tenderness documented at initial evaluation. No prior injury or treatment to the cervical spine is known. The treatment course followed a typical progression, with visit frequency tapering as symptoms improved and range of motion returned toward normal. Billing records reflect one charge per scheduled service, with no duplicate or repeated entries. At discharge, the patient was improving, with residual mild stiffness and a plan for independent home exercise.